

TM-002: TREATMENT ARC + SESSION 1 FULL SCRIPT

TM-002 (T.M., 22) | TRAUMA-001 | CSP Pro Bono | THERAPIST ONLY

TREATMENT ARC: 6-8 SESSION FLOW

All 6 presenting themes (trauma, self-esteem, confidence, trust, anxiety, letting go) share a single root: childhood trauma. The arc works from root outward to symptoms. EP result from Session 1 informs delivery lane for the program.

SESSION	FOCUS	THERAPEUTIC GOAL	KEY TECHNIQUE	BUILD ON
S1	Assessment + First Induction	Screen, determine EP, build rapport, establish safe space	Intake (L.O.V.E.), 36-Q suggestibility test, Theory of Mind, first induction, safe space visualization	Foundation
S2	The Root: Younger-Self Encounter	Access the core wound. Adult meets younger self in the Room. Identify the belief that was encoded.	Room Technique Phase 1-3 (build room, meet younger self, begin supportive dialogue). NOT regression. NEW safe room.	Safe space from S1
S3	Re-Parenting + Core Belief Reframe	Develop a fairer, more supportive response to earlier experiences	Room Technique Phase 4-5 (address unmet needs, build confidence). Core belief reframe.	Younger-self relationship from S2
S4	Integration + Anxiety Reduction	Carry the learning into adult behavior. Recalibrate threat detection.	Room Technique Phase 6 (integration). Anxiety dial/dimmer. Nervous system calming. Vagal toning.	Re-parenting from S3
S5	Trust + Assertiveness + Boundaries	Evaluate trust decisions. Install assertive voice. Boundary anchoring.	Circle of Trust visualization. Assertiveness chair. Boundary wall with gate (client controls).	Integrated perspective from S4
S6	Self-Hypnosis + Independence	Client runs the protocol alone. Reinforce all gains. Graduate.	Self-hypnosis teach (safe space + dial + affirmation). Relapse prevention. Future-pace.	All prior work
S7-S8	Contingency / Reinforcement	Only if needed. Address anything that surfaced. Reinforce.	Based on progress data.	As needed

Decision points:
After S1: EP informs Physical (direct language) vs Emotional (permissive inference) delivery style.
After S2: If younger-self encounter triggers strong catharsis, slow down. Add stabilization before S3.
After S3: If core belief has not shifted, repeat. Do not advance to integration prematurely.
After S4: If anxiety reduction is incomplete, reinforce before trust work in S5.
After S5: If client is self-sufficient, graduate at S6. If not, S7-S8 available.

CLINICAL RULES:
- Visit the past briefly to identify the encoded belief. Reframe it in present tense. Move forward. Do not loop. The goal is behavior change in the present.
- Check expectations at every session: "What are you hoping will happen today?"
- Younger-self work is an optional component. Ask: "Does this approach feel useful to you?" Declining does not change access to treatment.
- Do not interpret images or feelings in trance as factual memories. Uncertain material should be approached without confirmation or dismissal.
- If client meets criteria for PTSD, consider referral to CPT/PE/EMDR specialist. Hypnotherapy complements but should not displace evidence-based trauma treatment.

SESSION 1: FULL SCRIPT

[Client: T.M., 22, childhood trauma, 6 presenting themes. CSP pro bono. Prior psychotherapy. Requested male therapist. EP: TBD. This session is ASSESSMENT ONLY. No deep trauma work. No regression. Build the foundation.]

PART 1: RAPPORT + INTAKE (20-25 MIN)

Warm-Up (2-3 min, human connection first)

T.M., welcome. Good to see you. Before we get into anything clinical, how was your day? What have you been up to?

[2-3 minutes of normal conversation. Ask about his week, his interests, anything non-clinical. This is not small talk. This is the first building block of trust. He requested a male therapist for comfort. Match his energy. Be direct but warm. Do not perform "therapist mode" yet.]

Good. Thank you for making the time tonight. I know it takes a certain kind of courage to try something new, especially when you have already been through therapy before.

Session Frame (set expectations for tonight)

Here is how tonight works. This is our intake session. Think of it as the blueprint session. We are going to do four things.

First, we talk. I want to hear what brought you here, in your own words. No rush, no agenda.

Second, I ask some screening questions. Standard for every client, but especially important given what you have shared in your intake form. These are direct questions and I need honest answers so we can work safely together.

Third, I run a suggestibility assessment. It is not a test you pass or fail. It tells me how your mind processes information so I can tailor everything we do to how your brain works best.

Fourth, if you are comfortable, we do a first relaxation experience. This is optional. If at any point you want to stop, we stop. No questions asked.

Sound good?

L.O.V.E. Intake (Listen, Observe, Validate, Explore)

Tell me what brought you here. In your own words. Take your time.

[LISTEN. Do not interrupt. Let him tell the full story. Note the exact words he uses for each theme. His language reveals his subconscious framing. Watch for: body language shifts, vocal tone changes, eye contact patterns, emotional loading on specific words. Note which theme he leads with. That is usually what matters most right now.]

[If he goes quiet or seems unsure how much to share: "You can share as much or as little as you are comfortable with tonight. There is no pressure to cover everything in the first session."]

Thank you for sharing that. That takes courage, especially with someone new.

[VALIDATE. Do not minimize. Do not say "I understand" unless you genuinely do. Better: "That makes sense" or "Thank you for being honest about that." Do not rush to fix or reframe yet.]

I want to make sure I heard you correctly. You have been working on this in psychotherapy, and it has helped significantly. But there are some pieces that are still active. The self-esteem, the trust, the hypervigilance, the feeling of not being comfortable in your own skin. Those are still running in the background despite the insight you have gained. Is that accurate?

[REFLECT back. Let him correct you. His corrections tell you what matters most. If he corrects a word, use HIS word from now on.]

Can you give me a specific recent example? A situation in the last week or two where you noticed the anxiety or the hypervigilance showing up?

[EXPLORE. Get a concrete behavioral example. This grounds the work in real life. Also reveals which theme is most active RIGHT NOW. His answer may reshape the treatment arc priorities.]

And one more. When the anxiety is running the show, as you described it, where do you feel that in your body? Is there a physical sensation that goes along with it?

[Body awareness question. If he can identify a physical location (chest tightness, stomach knot, jaw clenching), that becomes a therapeutic target and a progress marker. If he cannot locate it physically, that is also data. It suggests he is more cognitively oriented. Adjust induction approach accordingly.]

Checking Expectations

Before we go further, I want to ask you something important. What are you expecting from hypnotherapy? What do you think will be different about this compared to the psychotherapy you have done?

[IMPORTANT. His answer tells you what matters most and what to prioritize. Understanding and change are not mutually exclusive. Listen for what he wants to be different. If he has unrealistic expectations (one-session fix, memory recovery, mind control), address them honestly now.]

What do you already understand about these reactions, and what would you like to be different in how you feel or respond?

And what are you hoping hypnotherapy will add to the work you have already done?

[His answer to this last question is the most important thing he says tonight. It becomes the treatment frame. Write it down verbatim.]

That is a great answer. Let me tell you how I think about it, and you tell me if it resonates.

The therapy you have done already made real progress. You said so yourself. That work gave you understanding, coping tools, and the ability to talk about what happened. Those are genuine gains. What you are describing now, the reactions that are still there despite that understanding, those often involve patterns that were encoded very early, before you had words for them. They can be harder to reach through conversation alone.

What we do here is work with a deeply relaxed state where your mind is more open to experiencing things differently. Not just understanding them, but feeling them shift. Think of it as adding an experiential layer to the insight you already have. We are not replacing what therapy gave you. We are building on it.

Does that make sense? Does that match what you are looking for?

[If yes: aligned. If no: explore further. Do not proceed until there is genuine alignment on what this process is and is not.]

PART 2: CRITICAL SCREENING (5-10 MIN)

[DO NOT SKIP ANY OF THESE. Childhood trauma requires thorough screening before any trance work. Ask directly. Do not soften. Record all answers.]

Before we go any further, I need to ask you some screening questions. These are standard for every client, but especially important given what you have shared. I need honest answers so I can make sure we work safely together.

Question 1: Active Professional Care

Are you currently seeing a therapist, psychologist, or psychiatrist? Are you in active treatment right now?

[If YES: get name, type of care, how often. Coordinate, do not conflict. If psychiatrist: check medications immediately. If NO: note that psychotherapy was past, not current. Ask when it ended and why.]

Question 2: Medications

Are you currently taking any medications? Anything for anxiety, depression, sleep, or anything else?

[If anti-psychotic: STOP. Refer to licensed MHP. Do not proceed with hypnosis. If anti-anxiety or anti-depressant: proceed with awareness. Note medication name and dosage. If recreational substances: note frequency. Do not judge.]

Question 3: Safety

I need to ask this directly, and I appreciate your honesty. Have you ever had thoughts of harming yourself or ending your life?

[If ACTIVE suicidal ideation: STOP the session. Provide 988 Suicide and Crisis Lifeline (call or text 988). Do not proceed with hypnosis. Refer to licensed MHP. If PAST ideation, now resolved: note it, proceed with caution. Ask: "When was the last time? What helped you through it?"]

Question 4: Trauma Scope

You mentioned experiences from when you were young. You do not need to share details today. But can you help me understand the general scope? Was this a single event, or something that happened over a period of time?

[Pause. Let him answer at his own pace.]

And without needing to go into specifics, did it involve physical harm, emotional harm, or both?

[Scope the trauma WITHOUT forcing disclosure. Session 1 is assessment, not processing. You need to know: single vs repeated, physical vs emotional vs both, who was involved (family, outside, authority figure). This determines the intensity and pacing of future sessions. Do NOT push for details he is not ready to share.]

Question 5: Triggers

Is there anything I should be careful about during our work together? Words, topics, types of imagery, or situations that tend to trigger strong reactions for you?

[ESSENTIAL for trance work. If he says "closed spaces" do not use confined visualizations. If "authority figures" adjust your tone and positioning. If "physical touch imagery" avoid body-contact language in progressive relaxation. If "darkness" use light-filled safe space imagery. Write these down. Refer to them before every future session.]

Question 6: Panic and Dissociation

Do you experience panic attacks? And when you get very stressed or upset, do you ever feel like you are watching yourself from the outside, or like things are not quite real?

[Panic: if yes, adjust induction. Use open-space imagery, not enclosed. Include grounding anchors throughout. Give him the exit signal. Dissociation: if yes, this is critical data. Dissociative clients require extra grounding in trance. Use body-awareness cues ("feel your feet, feel the chair") frequently. Monitor for blank staring or unresponsiveness during trance.]

Thank you. Those are the hard questions and you handled them well. Everything you share stays confidential, with two exceptions. If there is immediate danger to you or someone else, or if there is abuse of a child or elder, I have a legal obligation to report. Otherwise, this stays between us.

One thing to be aware of, because this is through the community service program: I may discuss our session with a clinical supervisor for educational and quality assurance purposes. Your identity is always protected. I would only use your initials. Is that okay with you?

[Get verbal acknowledgment of supervision disclosure. Note it.]

PART 3: SUGGESTIBILITY TEST (10-15 MIN)

Now I want to understand how your mind processes information. This is not a test you pass or fail. It is a diagnostic tool that tells me how to communicate with your subconscious most effectively.

There are two main processing styles. Physical and Emotional. Physical processors respond best to direct, literal, specific language. When I say "your hand IS rising," it rises. Emotional processors respond best to inference and suggestion. When I say "you may notice a sense of warmth," the warmth appears on its own. Everyone is a mix of both. I need to know your mix so I can tailor everything to how your brain is wired.

I am going to read you thirty-six statements. For each one, answer A or B based on which resonates more. There is no right or wrong answer. Go with your first instinct.

[Administer the 36-question HMI suggestibility test. Score it. Determine Physical/Emotional percentage. Record the exact split. This informs delivery style for the entire treatment arc.]

Alright, let me score this.

[Score and announce result.]

IF PHYSICAL (50%+ Physical):

"You scored [X]% Physical, [Y]% Emotional. That means your subconscious responds most effectively to direct, specific language. When I give you a suggestion, I will be literal and concrete. 'Your hand IS rising. Your body IS relaxing.' No ambiguity. Your mind takes direct instructions and runs with them. That is a strength. It means we can be very precise in our work."

IF EMOTIONAL (50%+ Emotional):

"You scored [X]% Emotional, [Y]% Physical. That means your subconscious works through inference and feeling. Instead of me telling you what to experience, I offer possibilities and your mind fills in what is right for you. 'You may notice a sense of warmth... you might feel a shift...' and your subconscious takes it from there. That is a strength. It means your mind is naturally creative in how it processes therapeutic suggestions."

IF BALANCED (within 10% of 50/50):

"You scored roughly balanced between Physical and Emotional. That gives us flexibility. Some suggestions will be direct, some will be more open-ended. Your mind will respond to both. We will find the right mix as we work together."

Any questions about that?

PART 4: THEORY OF MIND + EDUCATION (10-15 MIN)

Conscious vs Subconscious

Let me explain how this process works and how it connects to what you have already experienced in therapy.

You have two processing systems. The conscious mind is what you are using right now. It analyzes, reasons, makes decisions. It handles about 12 percent of your mind's total processing. The other 88 percent is the subconscious. It runs automatically. Your heartbeat, your breathing, your emotional reactions, your habits, your core beliefs about yourself and the world. All subconscious.

When you were a kid, experiences got encoded directly into that 88 percent. No filter. No adult analysis. Just raw emotional encoding. The beliefs that were formed, things like "I am not safe" or "I cannot trust people close to me" or "I have to stay on guard," those were not thoughts you chose. They were patterns your mind created to protect you. And they did protect you. They kept you alert when you needed to be alert.

The therapy you have done already made real progress. You said so yourself. That work gave you understanding, coping tools, and the ability to talk about what happened. Those are genuine gains, and we are going to build on them.

What you are describing now, the reactions that are still there despite that understanding, those often involve patterns that were encoded very early, before you had words for them. They can be harder to shift through conversation alone because they do not live in the conversational part

of the brain. They live in the experiential part.

What we do in hypnotherapy is work with a deeply relaxed state where your mind is more open to experiencing things differently. Not just understanding a pattern, but feeling it shift. We are adding an experiential layer to the insight you already have.

The Nervous System (fight-flight-freeze)

Here is another way to think about it. Your nervous system has a threat detection system. It was designed to protect you. When you were a kid and things were not safe, that system went on high alert. It learned to be hypervigilant because hypervigilance kept you safe. That was intelligent adaptation. Your brain did exactly what it was supposed to do.

The challenge is that it never fully recalibrated. You are 22 now. Your circumstances have changed. But the detection system is still running at the sensitivity level it was set to years ago. That is not a character flaw. That is not weakness. That is a protective program that did its job and has not received the update that the situation has changed.

Part of what we are going to do is help that system learn that it can adjust the sensitivity. Not turn off. You still need it. You need appropriate awareness of your environment. But right now the dial is set very high. We want to bring it to a level that matches your actual circumstances. Appropriate awareness, not hypervigilance.

Does that resonate with your experience?

[Let him respond. His response tells you how well the frame landed. If he says "that's exactly it" you have buy-in. If he pushes back, explore what does not fit.]

AI Disclosure

One more thing before we begin the relaxation. I want to be transparent about how I prepare. I use AI-assisted tools to research clinical approaches, prepare session materials, and organize notes. All the therapeutic decisions, what techniques to use, how to adapt them for you, how to pace our work, those are mine. The AI is a preparation tool, not a therapist. I mention this because transparency matters to me, and I want you to know exactly how I work.

Any questions before we move into the relaxation?

PART 5: FIRST INDUCTION + DEEPENING + SAFE SPACE (15-25 MIN)

- TRAUMA-SPECIFIC RULES:**
- NO age regression. No "go back to a time when..."
 - NO forced imagery. If he resists a visualization, follow his lead.
 - Include permission language: "with your consent," "only as far as comfortable"
 - Build a safe space that HE designs, not you. NEW space, not a childhood place.
 - Give him the exit signal BEFORE starting.
 - If abreaction: deepen trance, guide to safe space, use grounding language, do NOT count out abruptly.
 - PHS wording: add "safely and comfortably."
 - Monitor for dissociation: blank staring, unresponsiveness, sudden stillness. If observed, ground immediately.

Consent + Setup

Are you comfortable proceeding with the relaxation exercise? There is no pressure. We can save this for next session if you prefer.

[Get explicit verbal consent. If he hesitates, do not push. "We can absolutely wait. There is no timeline here."]

Good. Here is what is going to happen. I am going to guide you into a state of deep relaxation. You will be aware the entire time. You can hear me, you can think, you can open your eyes whenever you want. You are in control throughout. All I am doing is helping your conscious mind step to the side temporarily so we can work more directly with the subconscious.

This is not sleep. It is not unconsciousness. It is focused relaxation. Think of it like that state right before you fall asleep, when you are aware but your body is completely at rest.

If at any point you want to pause or come back, just raise your right index finger. That is your signal. I will bring you back immediately. No questions asked. Okay?

Good. Find a comfortable position. Take a deep breath in through your nose. Hold it for a moment. And exhale slowly through your mouth. One more. Deep breath in. And exhale. Good.

IF PHYSICAL EP: ARM-RAISING INDUCTION

[Use the Physical induction sequence below]

IF EMOTIONAL EP: EYE-CLOSURE INDUCTION

[Use the Emotional induction sequence below]

PHYSICAL INDUCTION: Arm-Raising

Raise your right arm straight out in front of you. Palm facing your face. Spread your fingers wide. Focus your eyes on your middle finger. Stare at it. Do not look away.

Take a deep breath in. As you exhale, notice your fingers. They are beginning to move. Slowly separating. Spreading apart. As if a gentle magnetic force is pulling them wider. You may or may not feel it yet. But watch. They are moving.

And as those fingers spread, your hand is beginning to drift. Moving toward your forehead. Slowly. Steadily. Like a magnet is pulling it closer. You do not need to push it. Just allow the drift. Your subconscious is moving your hand now.

With every breath, closer. Your eyelids are getting heavier. The closer your hand gets, the heavier your eyes become. Your hand is almost there. Closer. Closer.

The moment your hand touches your forehead, let your eyes close and nod your head.

[Wait. Do not rush. If the hand stalls, deepen the breathing: "Take another deep breath. And as you exhale, the hand moves closer." Time the full induction for post-session notes.]

Good. Eyes closed. Hand on forehead.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis, with your permission, you will enter this state safely and comfortably, quickly, calmly, and deeply, and the physical body will relax.

Deep Sleep.

[First PHS installed. Note the time.]

EMOTIONAL INDUCTION: Eye-Fixation + Closure

Find a spot on the ceiling or the wall above your eye line. Something you can focus on comfortably. Fix your gaze on that spot. Do not look away.

As you stare at that spot, you may notice your vision beginning to soften. The edges may blur slightly. The spot may seem to shimmer or shift. That is normal. It means your conscious mind is already beginning to step to the side.

And you may notice your eyelids growing heavier. Heavier with each breath. Not forcing them closed. Just allowing the heaviness to build. Like a gentle weight resting on each eyelid.

When they feel ready to close, allow them to close. There is no rush. Your subconscious will decide the right moment.

[Wait for eye closure. If eyes do not close after 60 seconds: "Allow your eyes to close now. Just let them fall shut."]

Good. Eyes closed. Settling in.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis, with your permission, you will allow yourself to enter this state safely and comfortably, at your own pace, and the body and mind will settle.

Deep Sleep.

5-0 Count Deepener (both lanes)

Hand down to your lap. Gently. Comfortably.

Five. Going down now. Deeper with every number. Your body remembers how to relax. It has done this a thousand times before sleep. This is no different.

Four. Physically letting go. Every muscle fiber standing down. Shoulders dropping. Jaw softening.

Three. Deeper. The conscious mind is stepping back. The subconscious is coming forward. You are safe here.

Two. Almost there. Comfortable. Supported. Nothing to do. Nowhere to be.

One. Deep. Quiet. Still.

Zero. Deep Sleep.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis, with your permission, you go deeper, safely and comfortably, and the physical body relaxes.

[PHS repeated. Second installation.]

Hand-to-Forehead Challenge (Physical EP only)

[PHYSICAL EP ONLY. Skip for Emotional EP. Replace with additional progressive relaxation.]

Your hand is still connected to your forehead. That connection is solid. Like two magnets pressed together. The harder you try to pull it away, the stronger it presses. Try now. Try to pull your hand away. And notice that it will not move. It stays. Locked. Solid.

[Brief test, 3-5 seconds.]

Good. Stop trying. Five. Four. Three. Two. One. Zero. Deep Sleep.

Hand down gently to your lap. Deeper now. Twice as deep.

Each and every time I suggest Deep Sleep, for the purpose of hypnosis, with your permission, you go deeper, safely and comfortably.

[PHS repeated. Third installation.]

Reactional Deepener (2 cycles, both lanes)

In a moment I am going to ask you to open your eyes. When I say Deep Sleep, they close again and you go twice as deep. Ready?

Open your eyes.

[Wait 2 seconds.]

Deep Sleep. Twice as deep. Eyes closed. Sinking down.

One more. Open your eyes.

[Wait 2 seconds.]

Deep Sleep. Twice as deep. Deeper than before. Comfortable. Safe.

Each and every time, for the purpose of hypnosis, with your permission, quickly, calmly, deeply, safely and comfortably, and the physical body relaxes.

[PHS repeated. Fourth installation. Note: only 2 cycles for first session with trauma client. Do not overload.]

Progressive Relaxation (both lanes, delta-paced)

[Delta pacing: 2-second pauses between phrases. Slow. Rhythmic. Match your breathing to his. This is where the body fully lets go.]

Now focus on the very top of your head. [Pause.] Allow any tension there to soften. [Pause.] Like warm water slowly spreading across your scalp. [Pause.]

Your forehead smooths. [Pause.] The muscles around your eyes release. [Pause.] Your jaw unclenches. [Pause.] Lips slightly parted. [Pause.] The entire face letting go of whatever it has been holding. [Pause.]

Neck softens. [Pause.] Feel the muscles on either side of your spine release. [Pause.] Shoulders drop to their natural resting position. [Pause.] Not held up. Not braced. Just resting where gravity places them. [Pause.]

Arms heavy. [Pause.] Forearms. [Pause.] Wrists. [Pause.] Hands open. [Pause.] Fingers gently curled. [Pause.] All the tension draining down through your fingertips. [Pause.]

Chest opens. [Pause.] Breathing deepens on its own. [Pause.] You do not need to control it. [Pause.] Your body knows how to breathe. [Pause.] Let it breathe for you. [Pause.]

Core relaxes. [Pause.] Lower back releases. [Pause.] Hips soften. [Pause.] All the protective tension your body has been carrying, all day, all week, maybe all year, letting go. [Pause.]

Thighs heavy. [Pause.] Knees. [Pause.] Calves. [Pause.] Ankles. [Pause.] Feet warm and grounded. [Pause.] Connected to the floor beneath you. [Pause.] Stable. Supported. Safe. [Pause.]

Every system in your body shifting into rest mode now. [Pause.] The alarm system turning down. [Pause.] Not off. Just down. [Pause.] To a level that matches this moment. [Pause.] This moment is safe. [Pause.]

Five. [Long pause.] Four. [Long pause.] Three. [Long pause.] Two. [Long pause.] One. [Long pause.] Zero. Deep Sleep.

Each and every time, for the purpose of hypnosis, with your permission, safely and comfortably, quickly, calmly, deeply, and the physical body relaxes.

[PHS repeated. Fifth installation.]

Safe Space Building (THE ROOM - client designs)

[This safe space becomes THE ROOM for younger-self work in Sessions 2-3. The client MUST design it. NOT a childhood place. A brand new space that belongs to the adult. Engage all senses. Let him choose. If Physical EP, prompt with specific sensory questions. If Emotional EP, offer possibilities and let him discover.]

Now I want you to create something. A space. A room that belongs entirely to you. No one else has ever been here. No one can enter without your invitation. This is a new space. Not a memory. Not a place from your past. Something your mind is building right now, just for you.

IF PHYSICAL: "What does this room look like? Tell me. What color are the walls? What is on the floor? Is there a window? What do you see through it?"
[Wait for responses. Use his answers to build detail.]

IF EMOTIONAL: "Allow this space to take shape on its own. You may notice colors appearing... textures... a sense of the space around you. Let it form at whatever pace feels right."

Notice the temperature in this room. Is it warm? Cool? Just right? [Pause.] What does the air feel like? [Pause.]

Are there sounds? Maybe silence. Maybe something gentle in the background. Whatever feels right for this space. [Pause.]

Is there a scent? Something clean, something natural, something comforting? [Pause.]

Is there a place to sit? A chair, a couch, a cushion on the floor? Find the spot that feels exactly right and settle into it. [Pause.]

Feel how this space holds you. You decide what enters and what stays outside. You control the door. You control the windows. You control every aspect of this environment. It is entirely yours.

This space is being installed right now. It will be here whenever you need it. In session, between sessions, in the middle of the night, in the middle of a difficult conversation. Close your eyes, take one breath, and you are here.

Subconscious mind, when this room is fully established and you feel completely safe and comfortable inside it, allow a nod to happen.

[Wait for nod. This is the first ideomotor check. If no nod comes after 30 seconds, gently: "Take your time. There is no rush. When it feels ready, let it happen." If still no nod after another 30 seconds, proceed. Some clients do not give strong ideomotor signals on the first session. That is okay.]

Good. That is your room now. Installed. Permanent. Available anytime.

Grounding Anchor (triple-fire installation)

Before we come back, I want to give you a physical tool. Something you can use anytime, anywhere, without anyone knowing.

Press your thumb and index finger together on your left hand. Feel that pressure. Really feel the contact between the two fingers. That physical sensation is now being linked to this feeling. This safety. This room. This calm.

[First fire.]

Now release. [Pause.] And press them together again. Feel the calm deepen. Feel the room around you become more vivid. The anchor is strengthening. [Pause.]

[Second fire.]

Release. [Pause.] One more time. Press. And this time, as you press, take a slow breath in. And as you exhale, feel every bit of tension in your body drain out through your feet. The anchor. The breath. The calm. Linked together now. Permanently.

[Third fire. Triple installation makes the anchor significantly stronger than a single fire.]

Subconscious mind, when this grounding anchor is fully installed and ready to use, allow a nod to happen.

[Wait for nod.]

Good. Anytime the anxiety shows up, anytime the hypervigilance kicks in, press your thumb and finger, take one breath, and the calm returns. It may not be as deep as right now. But it will be there. A physical anchor you carry with you.

Post-Hypnotic Suggestion (between sessions)

One more thing before we come back. Between now and our next session, you may notice moments of unexpected calm. Moments where the volume on the anxiety is just a little bit lower than usual. Moments where you catch yourself feeling at ease and think, "that is different." When those moments happen, notice them. Do not analyze them. Just notice. Each one is evidence that the work we did tonight is active. The subconscious is processing.

PART 6: COUNT OUT (GENTLE, GROUNDING)

[Gentle count out. NOT rapid 1-2-3-4-5. Trauma clients need grounding, not abrupt awakening. Re-establish body awareness before opening eyes.]

In a moment I am going to bring you back. You will bring with you everything we installed tonight. The safe room. The grounding anchor. The post-hypnotic suggestion. All permanent. All yours.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis, with your permission, you will enter this state safely and comfortably, quickly, calmly, and deeply. That suggestion is sealed now.

[Final PHS seal before count out.]

Zero. Beginning to come back. Slowly. At your own pace.

One. Feeling your body. Becoming aware of your feet on the floor. The weight of your body in the chair. The temperature of the room.

Two. Taking a deep breath in. Energy returning. Feeling refreshed. Not groggy. Clear.

Three. More aware now. Hearing the sounds around you. The hum of the room. Feeling present.

Four. Almost back. Fingers and toes moving gently. Eyes ready to open when you are ready.

Five. Eyes open. Fully alert. Grounded. Present. Safe. Welcome back.

Take a moment. There is no rush.

PART 7: POST-TRANCE DEBRIEF + PHS VERIFY + BASELINES

Debrief (do not skip)

How do you feel?

[Let him answer. Give him 30-60 seconds. First trance can be emotional, especially for trauma clients. If he tears up, that is normal: "That is a completely normal response. Take your time."]

What did you notice during the relaxation? Were there any images, sensations, or feelings that stood out?

[Do NOT interpret his experience. Do not say "that means..." Just listen and acknowledge. If he describes his safe room, that is excellent data. Note the details for future sessions.]

How deep did you feel you went? If zero is wide awake and ten is the deepest possible, where would you put yourself?

[Record self-reported depth ___/10. This is his subjective experience, not a clinical measurement. But it is useful for comparison across sessions.]

PHS Verify (finger-spread test)

Let us do a quick test to see how the suggestion installed. Raise your arm. Palm facing you. Focus on your middle finger. Take a breath.

Fingers separating. Hand moving toward your face. Eyelids heavy. Closer. When your hand touches, nod. Deep Sleep.

[Time the response. Compare to induction duration. Faster = stronger PHS installation. Count out 0-1-2-3-4-5. Note: for trauma clients, do a gentle count out even from PHS verify.]

Good. Welcome back. The post-hypnotic suggestion is installed and active. Each session, that response will get faster. That acceleration is one of the ways we measure progress.

SMART Goal Baselines

Before we wrap up, I want to get some baseline numbers. These are subjective. No wrong answers. Just where you are right now, tonight.

On a scale of 1 to 10, where 10 is the best it could be:

How comfortable do you feel being yourself right now? Just being in your own skin?

[Record: Self-esteem baseline ___/10]

How easy is it for you to set boundaries or say no when you need to?

[Record: Assertiveness baseline ___/10]

On a typical day, how much does anxiety affect your decisions and behavior? One is barely there, ten is it runs everything.

[Record: Anxiety baseline ___/10 (high = more anxiety)]

How much do you trust the people closest to you? One is very little, ten is completely.

[Record: Trust baseline ___/10]

How often do you feel genuinely at ease, just existing without being on guard?

[Record: Comfort baseline ___/10]

And how much does the past still influence how you feel and react in the present? One is barely at all, ten is it shapes everything.

[Record: Past influence baseline ___/10 (high = more past control)]

Thank you. Those numbers give us a clear starting point. We will check them again periodically so you can see the movement.

PART 8: HOMEWORK + NEXT STEPS

Here is your homework between now and our next session. Two things. Just two.

First: the grounding anchor. Thumb and index finger, one deep breath. Use it whenever the anxiety shows up or the hypervigilance kicks in. Use it before a difficult conversation. Use it when you cannot sleep. The more you use it, the stronger it gets. It is a muscle you are building.

Second: at some point this week, sit quietly for two or three minutes, close your eyes, and visit the room you built tonight. Walk around it. Notice the details. Maybe add something. A book. A blanket. Whatever feels right. Strengthen that space. It is going to become an important tool in our work together.

That is it. Two things. No journals, no tracking sheets, no complicated protocols. Just the anchor and the room.

[Keep homework simple for Session 1. Trauma clients with therapy history have homework fatigue risk. Too much = overwhelm = dropout. Build complexity gradually. If he asks "should I journal?" say: "Only if that feels helpful to you right now. It is not required."]

How are you feeling about everything we covered tonight?

[Let him respond fully. Listen for: relief ("I already feel different"), surprise ("that was not what I expected"), skepticism ("I'm not sure it worked"), emotional processing (quiet, reflective). All are normal and healthy responses.]

You did really well tonight. The fact that your subconscious accepted the safe space and the grounding anchor on the first session tells me your mind is ready for this work. That is not always the case. You showed up, you were honest, you let yourself experience something new. That is a strong start.

For our next session, we will go deeper. We will use that safe room you built and begin the therapeutic work on the core issues you described. But tonight was the foundation, and it is solid.

Let us schedule session two. I would suggest one to two weeks from now. What works for your schedule?

[Schedule Session 2. Provide Calendly link: calendly.com/jeeth-vrishihypno/90min]

Last thing. If between now and our next session anything comes up that feels overwhelming, if a memory surfaces or the emotions feel too intense, you have two options. First, use the anchor. Thumb and finger, one breath. Second, reach out to me. You have my email. And if it is ever urgent, call or text 988. That is the national crisis line. Available 24 hours. You are not alone in this.

Thank you for trusting me with this, T.M. I will see you at session two.

- [After client disconnects:*

 - *Complete SOAP notes immediately while fresh*
 - *Record EP result, PHS time, self-reported depth, all 6 baseline numbers*
 - *Note his safe room description in detail (you will reference it in S2)*
 - *Note any triggers he identified*
 - *Note screening answers (medications, care, safety, trauma scope, dissociation)*
 - *Update TM-002.json with session data*
 - *Generate AVS: python scripts/generate_client_docs.py data/clients/TM-002.json*
 - *Log CCH hours to HMI*
 - *Reflect: what went well? What would you adjust for S2?]*